

CarePlus Orthopedic & Sports Medicine Center

Fictional Sample Medical Report - For Software Testing Only

Report Type	Knee Injury Final Recovery Evaluation	Report ID	ORTH-KNEE-2026-0068
Date of Examination	2026-09-20	Examining Physician	Dr. Sameer Kulkarni, MS Orthopedics
Referring Physician	Dr. Kavya Rao	Facility	CarePlus Orthopedic Center
Patient Name	Arjun Mehta	Patient ID	CP-PT-10482
Age / Gender	32 / Male	Affected Joint	Right Knee

CHIEF COMPLAINT

Patient presents for final recovery evaluation of right knee ligament injury. Patient reports near-complete improvement with no resting pain, no visible swelling, and only rare mild tightness after prolonged high-intensity activity.

HISTORY OF PRESENT ILLNESS

The patient has completed an extended conservative management program including bracing, activity modification, progressive strengthening, supervised physiotherapy, proprioception training, and home exercise compliance. He reports normal walking, comfortable stair climbing, improved squatting tolerance, and no instability episodes.

INJURY DETAILS

Previous Injury Type	Partial ACL sprain with resolving Grade I MCL sprain
Original Date of Injury	2026-04-07
Pain Score	1/10
Swelling Grade	None to trace only after heavy activity
Weight Bearing	Full weight bearing without discomfort
Range of Motion	0 degrees to 140 degrees
Knee Flexion	Full functional flexion
Knee Extension	Full and painless

PHYSICAL EXAMINATION

Inspection: No visible swelling; normal knee contour.

Tenderness: No medial joint line tenderness.

Lachman Test: Negative.

Anterior Drawer Test: Negative; no significant laxity.

McMurray Test: Negative.

Varus Stress Test: Negative.

Valgus Stress Test: Negative.

Patellar Tracking: Normal.

Neurovascular Status: Intact.

Gait: Normal, non-antalgic gait pattern.

VITAL SIGNS

Parameter	Value
Blood Pressure	118/76 mmHg
Heart Rate	68 bpm
Temperature	98.1 F
SpO2	99%

FINAL FOLLOW-UP MRI RIGHT KNEE FINDINGS

1. Near-complete healing of previous partial anterior cruciate ligament sprain.
2. Medial collateral ligament sprain has resolved clinically with only minimal residual signal change.
3. Joint effusion has nearly resolved.
4. No new meniscal tear detected.
5. No fracture, bone marrow edema, or cartilage defect.
6. Articular cartilage remains preserved.
7. Overall alignment and patellar tracking remain normal.

MEASURED VALUES

Measurement	Current Value	Status
Joint Effusion	1 ml estimated	Improved
ACL Fiber Disruption	Approximately 2% residual signal change	Improved
MCL Sprain Grade	Resolved Grade I sprain	Improved
Medial Joint Space	5.0 mm	Stable
Lateral Joint Space	5.3 mm	Stable
Quadriceps Strength	5/5	Normal
Hamstring Strength	5/5	Normal
Knee Stability Score	95/100	Improved
Functional Mobility Score	94/100	Improved
Single-Leg Balance Test	45 seconds without instability	Improved

COMPARISON WITH PREVIOUS FOLLOW-UP REPORT DATED 2026-07-12

Metric	2026-07-12	2026-09-20	Change
Pain Score	3/10	1/10	Improved
Swelling Grade	Mild	None / trace only after heavy activity	Improved
Weight Bearing	Full with mild discomfort	Full without discomfort	Improved
Range of Motion	0 degrees-130 degrees	0 degrees-140 degrees	Improved
Joint Effusion	6 ml	1 ml	Improved
ACL Fiber Disruption	8%	2%	Improved
Quadriceps Strength	5/5	5/5	Maintained normal
Hamstring Strength	5/5	5/5	Maintained normal
Knee Stability Score	86/100	95/100	Improved
Functional Mobility Score	82/100	94/100	Improved
McMurray Test	Negative	Negative	Stable normal
Valgus Stress Test	Negative	Negative	Stable normal

DIAGNOSIS

Clinically recovered right knee partial ACL sprain with resolved Grade I MCL sprain. Excellent functional improvement noted with restored strength, stable ligament testing, near-complete MRI healing appearance, and no new structural abnormality.

TREATMENT PLAN

1. Continue home strengthening program 3 to 4 times weekly for maintenance.
2. Progress to sport-specific agility drills under physiotherapy supervision.
3. Begin gradual return to recreational running if no pain, swelling, or instability occurs.
4. Avoid sudden pivoting or competitive contact sport until final sports clearance.
5. Use knee sleeve only for confidence during early return-to-activity phase if needed.
6. Follow up only if pain, swelling, locking, giving-way, or instability returns.

MEDICATIONS

No regular pain medication required. Topical anti-inflammatory gel may be used only if mild post-exercise soreness occurs.

RECOMMENDATION

Patient shows excellent recovery with conservative management. Gradual return to activity is advised. Full sports clearance may be considered after completion of agility and sport-specific testing without symptoms.

PATIENT-FRIENDLY SUMMARY

Your knee has improved in nearly every tracked area. Pain is minimal, swelling has almost fully resolved, motion has improved, strength is normal, and the knee stability score is much better than the last report. Continue exercises and return to activity slowly. Stop and contact your doctor if swelling, pain, locking, or instability comes back.

Doctor Signature: Dr. Sameer Kulkarni
Consultant Orthopedic Surgeon
CarePlus Orthopedic & Sports Medicine Center